ABOUT THIS GUIDE

Evidence-based pregnancy information organized by trimester. A companion for every appointment and every question.

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ACOG No. 771 · AAP Guidelines · Standard prenatal care protocols

WELCOME

Congratulations.

You're pregnant. From this point forward, your body will do something extraordinary — growing a new life from a single cell. Some days you'll feel amazing. Others, you'll wonder what happened to your energy, your appetite, or your ability to stay awake past 8 PM.

All of it is normal. This guide walks you through what to expect, what to do about it, and what decisions are ahead — trimester by trimester.

40	3	25+
WEEKS	TRIMESTERS	TOPICS

Your Pregnancy at a Glance

1st Trimester	**Weeks 1–12**
2nd Trimester	**Weeks 12–28**
3rd Trimester	**Weeks 28–40**
First prenatal visit	**~Week 6**
Baby's first kicks	**~Week 23**
Glucose screening	**Weeks 24–28**
Full term	**Week 37+**

A NOTE ABOUT THIS GUIDE

Every pregnancy is different. This is not a substitute for your provider's care — it's a companion to help you know what to ask and what to expect.

SAFETY

Avoid & Limit

Avoid

- Smoking, alcohol, and recreational drugs — all increase risk of miscarriage, premature birth, and developmental problems. If you need help stopping, talk to your provider.
- High-mercury fish (shark, swordfish, king mackerel, tilefish)
- Raw meat, fish, or shellfish (sashimi, ceviche, raw oysters)
- Soft cheeses (brie, feta, blue, goat, camembert)
- Unpasteurized juice or milk
- Cat litter — toxoplasma risk. Your cat is fine; the litter box isn't.
- Hot tubs and saunas — risk of overheating and dehydration
- Aspirin, Motrin, Ibuprofen, Aleve, Advil — may cause bleeding or complications

Limit

- Caffeine: ≤200mg/day (about two 8oz cups)
- All fish including tuna: under 12oz/week
- Deli meats: heat until steaming before eating
- Artificial sweeteners: occasional use is considered safe

Safe

- Tylenol (acetaminophen) for pain
- Warm baths (not hot tubs)
- Cooked shellfish
- Pasteurized dairy and juice
- Gardening with gloves

KNOW THE SIGNS

When to Get Help

Your body will tell you when something needs attention. Don't wait.

1 Call Your Provider

- Any vaginal bleeding or spotting
- Fluid leaking from your vagina
- Severe headache or changes in vision
- Severe abdominal or pelvic pain
- Fever over 100°F
- Vomiting or diarrhea lasting 12+ hours
- Noticeable decrease in fetal movement
- Contractions 3–4×/hr before week 37

2 Go to the Hospital

- Contractions every 5 minutes
- Your water has broken
- Vaginal bleeding (more than spotting)
- Significant decrease in baby's movement
- Fever over 100°F with other symptoms

Save your provider's number and hospital number in your phone now. Post them where your family can find them.

TESTING TIMELINE

Your Screenings

When each test happens across your 40 weeks.

0 10 20 30 40

Blood Tests & Ultrasound — **Wk 6**
Blood type, Rh status, anemia, infections, diabetes markers. First ultrasound confirms viability.

Carrier Screening — **Wk 6**
Cystic fibrosis, Tay-Sachs, sickle cell. Mother tested first; father if positive.

NIPT optional — **Wk 10**
Blood screen for Down Syndrome. Over 99% accurate. No miscarriage risk.

NT Scan optional — **Wk 11**
Neck fold measurement. 60–80% detection for Down Syndrome.

AFP/Quad Screen — **Wk 15–20**
Blood test for genetic disorders and neural tube defects. Most accurate at 16–18 weeks.

Amniocentesis optional — **Wk 16–20**
>99% accurate. Miscarriage risk <3 in 1,000. Usually offered if screenings positive or age >35.

Anatomy Ultrasound — **Wk 18–22**
Detailed look at brain, heart, limbs, kidneys. Often reveals baby's sex.

Glucose Screening — **Wk 24–28**
1-hour blood test for gestational diabetes. 3-hour follow-up if elevated.

RhoGAM — **Wk 28**
Injection for Rh-negative mothers. Protects this and future pregnancies.

Group B Strep — **Wk 35–37**
Vaginal/rectal swab. Present in 20–35% of women. Antibiotics during labor if positive.

1st Tri · 2nd Tri · 3rd Tri · Optional

NUTRITION

Eating Well for Two

Daily servings your baby needs to grow. Most women gain 20–35 lbs during pregnancy.

Protein	**3×/day**
Chicken, fish, eggs, nuts, beans, soybeans	
Calcium	**3×/day**
Milk, yogurt, cheese, leafy greens, almonds, sardines	
Vegetables & Fruit	**4×/day**
Aim for variety and color	
Fats & Oils	**4×/day**
1 serving = 1 tbsp oil, butter, or peanut butter	
Grains	**6×/day**
Bread, cereal, rice, pasta	

EATING BY TRIMESTER

1st Trimester
Your baby needs very little. Focus on what you can keep down — crackers, fruit, cheese, yogurt. Prenatal vitamin daily. At least 6 glasses of water.

2nd Trimester
Balanced diet becomes critical. Your baby is growing rapidly and needs the full range of nutrients. Follow the daily framework above.

3rd Trimester
Appetite may decrease even as baby grows fastest. 5–6 smaller meals daily. Avoid carbonated drinks. Good sources: eggs, beans, nuts, yogurt, leafy greens.

YOUR PERSONAL GUIDE TO

Pregnancy

What to expect. What to eat. What to ask. What to decide. Organized by trimester, backed by evidence.

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1ST TRIMESTER

01 P8

Your Body & Baby

Weeks 1–12

What You May Experience

Fatigue
Your body is building a placenta, increasing blood volume, and supporting rapid cell division. That takes enormous energy.

Rest when you can — don't fight it. Naps aren't laziness, they're biology.

Nausea
Rising hormone levels affect your digestive system. It can hit any time of day, not just mornings.

Dry crackers before getting up. Small bites every 30 min. Ginger tea or chews. Prenatal vitamin at bedtime.

Tender Breasts
Hormonal changes can make your breasts swollen and sensitive — sometimes seemingly overnight.

A well-fitted, supportive bra makes a real difference.

Bloating & Constipation
Your digestive system slows down to absorb more nutrients for the baby.

Increase fluids, fresh fruits, and vegetables. Chewable papaya enzyme tablets after meals can ease gas.

Cramping
Mild, menstrual-like cramps are normal as your uterus expands.

Sharp or severe pain is not normal — call your provider.

Your Baby

6 **Beating heart**, early eye structures, and limb buds forming. About ½ inch long.

8 **Webbed fingers and toes.** All major organ systems developing.

12 **3½ inches.** Muscles, bones growing. Fingers separated. Genitals forming.

1ST TRIMESTER

01 P9

Your To-Do List

Weeks 1–12

- [ ] **Complete your prenatal lab work** — Blood type, Rh status, infection screening, and baseline health markers.
- [ ] **Discuss genetic testing with your provider** — Several options exist at different stages. Understand what's available, what insurance covers, and what the results can and can't tell you.
- [ ] **Ask about preterm birth risk** — About 1 in 10 babies are born early. Know your risk factors, the warning signs, and the plan if it happens.
- [ ] **Discuss blood pressure monitoring** — High blood pressure in pregnancy can lead to pre-eclampsia. Early monitoring makes a difference.
- [ ] **Call your insurance company** — Review what's covered for prenatal care, delivery, and newborn care before you need it.
- [ ] **Prioritize rest** — Your body is working harder than it feels. Sleep when you're tired. It's not laziness — it's biology.
- [ ] **Start or continue exercising** — Even 10 minutes of walking a day helps with fatigue, mood, and sleep quality.
- [ ] **Learn about cord blood preservation** — Your baby's umbilical cord contains stem cells used in transplant medicine for over 30 years. You'll need to decide before delivery. See Panel 14.

CALL YOUR PROVIDER

- Bleeding or severe abdominal pain
- Severe headache or fever over 100°F
- Vomiting or diarrhea lasting 12+ hours

2ND TRIMESTER

02 P10

Your Body & Baby

Weeks 12–28

What You May Experience

More Energy
Better sleep, returning appetite, and a noticeable lift in how you feel. This is often the most comfortable stretch of pregnancy.

Take advantage of this window — sign up for classes, prepare the nursery, enjoy how you feel.

Baby's Movement
Most women start feeling movement around week 23 — beginning as flutters and building to distinct kicks and rolls.

One of the most exciting milestones. By week 28, you'll begin tracking kick counts.

Varicose Veins
Increased blood volume and uterine pressure can cause veins in your legs to swell.

Wear support stockings. Elevate your legs when possible. Avoid crossing them.

Braxton Hicks Contractions
Mild uterine tightening 2–4 times a day is your body practicing for labor. Normal and not dangerous.

If contractions come 3–4 times in one hour, call your provider.

Anemia
If diagnosed, your iron levels need support. This is common and treatable.

Eat more meat, leafy greens, and beans. Take iron with juice, not milk. Increase fiber to offset constipation.

Your Baby

16 **Ears, eyelashes, and fingernails** are developing. Legs are getting longer. Fingers open and close.

20 **About 7 inches long.** Covered in fine hair called lanugo. Practicing sucking movements.

24 **Eye movements begin.** Lungs are producing surfactant — a substance essential for breathing air.

28 **About 10 inches.** Practicing breathing movements. Eyes open and close.

2ND TRIMESTER

02 P11

Your To-Do List

Weeks 12–28

- [ ] **AFP/Quad screen** — A blood test between weeks 15–20 that screens for genetic disorders and neural tube defects. Most accurate at 16–18 weeks.
- [ ] **Level 2 ultrasound** — Between weeks 18–22. A detailed evaluation of your baby's anatomy — brain, heart, limbs, and kidneys.
- [ ] **Start sleeping on your side** — Beginning at week 20. Right or left side is fine — the goal is staying off your back. A body pillow can help.
- [ ] **Glucose screening** — Between weeks 24–28. A one-hour blood test that screens for gestational diabetes.
- [ ] **RhoGAM injection** — At week 28 if you are Rh-negative. This protects your current and future pregnancies.
- [ ] **Begin kick counts** — Starting at week 28. When resting, you should feel at least 4 movements in one hour. Talk to your provider about their specific guidelines.
- [ ] **Decide on your labor support** — Partner, family member, friend, or doula. Think about who you want in the room.
- [ ] **Sign up for childbirth and newborn care classes** — Prepared childbirth, breastfeeding, and infant CPR. These fill up — register early.
- [ ] **Research cord blood preservation** — Your baby's umbilical cord blood contains stem cells that have been used to treat over 80 serious conditions. It's worth understanding your options before delivery. See Panel 14.

CALL YOUR PROVIDER

- Bleeding, leaking fluid, or changes in vision
- Noticeable decrease in fetal movement
- Pressure or pain 3–4 times per hour

3RD TRIMESTER

03 P12

Your Body & Baby

Weeks 28–Delivery

What You May Experience

Heartburn
Your baby pushes your stomach upward, causing acid reflux after meals or when lying flat.

Smaller meals, more often. Stay upright 30 min after eating. Lie on your right side. Antacids are safe.

Constipation & Hemorrhoids
Digestive slowdown and pelvic pressure make both common.

More fruits, vegetables, and water. Ask about a stool softener. OTC hemorrhoid remedies are available.

Back Pain & Leg Cramps
Extra weight shifts your center of gravity. Cramps tend to strike at night.

Bend at knees, not waist. For cramps, point toes toward belly and try ice. Prenatal massage and PT can help.

Swelling & Leaking
Fluid retention causes swollen feet/ankles. Bladder pressure can cause leaking when you laugh or cough.

Elevate legs. Increase water and protein. Kegels (50–100/day) help with leaking. Sudden severe swelling — call your provider.

Fatigue & Mood Changes
Interrupted sleep takes a toll on energy and emotions.

If you feel overwhelmed, anxious, or persistently sad, talk to your provider. Common and treatable.

Your Baby

28 **About 14 inches and 2 pounds.** Lungs are maturing. Sleep and wake cycles are developing. Hiccups are a normal sign of development.

34 **Most babies have settled head-down** — the position they'll likely stay in until delivery.

40 **18–22 inches long, 6–9 pounds.** Fully developed and ready to be born.

3RD TRIMESTER

03 P13

Your To-Do List

Weeks 28–Delivery

- [ ] **Group B Strep screening** — A vaginal and rectal swab between weeks 35–37. If positive, you'll receive antibiotics during labor to protect your baby.
- [ ] **Choose a pediatrician** — Interview candidates and confirm they accept your insurance. Do this before your due date.
- [ ] **Take a hospital tour** — Know the facility, where to park, and the fastest route from home.
- [ ] **Install the car seat** — You will not be allowed to leave the hospital without one properly installed.
- [ ] **Pack your hospital bag** — For you and for baby. Include your insurance card, ID, phone charger, and going-home outfits.
- [ ] **Discuss expanded newborn screening** — A panel of tests performed at birth that screens your baby for a range of serious but treatable conditions.
- [ ] **Finalize your birth preferences** — Discuss pain management, labor positions, and who will be in the room with your provider and support team.
- [ ] **Confirm your cord blood decision** — If you haven't enrolled yet, it's not too late — even days before delivery. Visit **stemcyte.com** or call **1-888-CORD-BLOOD**.

GO TO THE HOSPITAL

- Contractions every 5 minutes
- Your water has broken
- Vaginal bleeding
- Significant decrease in baby's movement

AT BIRTH

P14

Newborn Stem Cells

Your baby's cord blood and cord tissue contain stem cells used in transplant medicine for over 30 years — treating cancers, blood disorders, immune deficiencies, and metabolic conditions.

80+	500+	~5
CONDITIONS	CLINICAL TRIALS	MIN COLLECTION

YOUR OPTIONS

1 Private Storage
Cord blood and cord tissue stored exclusively for your family. Available if a medical need arises for the child or first- or second-degree relatives.

2 Public Donation
Donated to a public bank for any patient in need. Your family does not retain ownership. Available at hospitals with a public bank partnership.

3 Discard
Discarded as medical waste after delivery. Once gone, they cannot be recovered.

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Public & Private Cord Blood Banking · AABB · FACT · FDA Licensed